BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Blood count, kidney and liver function, glucose, thyroid tests and BRAF tumour status. Review autoimmune disease, previous immune reactions, transplantation, infection and all medicines. Confirm informed consent for this unlicensed melanoma dosing schedule and the local governance requirements.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	National Cancer Control Programme (NCCP) specifies ongoing monitoring for at least five months after the last dose. Immune effects can require longer follow-up, steroid tapering or lasting hormone replacement. Response scans and any further melanoma treatment follow the individual plan.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00551 v7a: current regimen source

THE WHOLE TREATMENT JOURNEY

Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma: treatment and checks

Advanced melanoma

Up to four combination infusions, 21 days apart, then nivolumab alone every two or four weeks as prescribed; four stamps record combination doses only.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00551	Blood count, kidney and liver function, glucose, thyroid tests and BRAF tumour status. Review autoimmune disease, previous immune reactions, transplantation, infection and all medicines. Confirm informed consent for this unlicensed melanoma dosing schedule and the local governance requirements.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	NCCP specifies ongoing monitoring for at least five months after the last dose. Immune effects can require longer follow-up, steroid tapering or lasting hormone replacement. Response scans and any further melanoma treatment follow the individual plan.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00551	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00551	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 Combination induction

Up to four doses

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00551

2 Nivolumab alone

Ongoing review

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00551

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42